KMTC/QP-08/EPS

KENYA MEDICAL TRAINING COLLEGE
FACULTY OF NURSING
DEPARTMENT OF NURSING

FINAL QUALIFYING EXAMINATION
FOR
DIPLOMA IN COMMUNITY HEALTH NURSING

PAPER: MIDWIFERY

DATE: 22nd January 2020 **TIME: 3 HOURS (9:00AM – 12:00 NOON)**

INSTRUCTIONS

1. This paper consists of:
   - Section 1 (40 Multiple Choice Questions)
   - Section 2 (8 Short Answer Questions)
   - Section 3 (1 Long Answer Question)
2. Attempt **ALL** Questions
3. Write the EXAMINATION **NUMBER** given on all the answer sheets provided and on the question paper.
4. Ensure that all examination answer scripts are handed in at the end of the examination
5. Ensure you sign the examination register provided

EXAMINATION NUMBER .....

SECTION 1

MULTIPLE CHOICE QUESTIONS (40 MARKS)

1. The statement last menstrual period means
   a) The first day of the last menses that the woman had
   b) The last day of the last menses that the woman had
   c) The last month the woman had menses
   d) The monthly period that occurred the month the woman attended the antenatal clinic
2. The part of the fetus that lies in the lower pole of the uterus in a breech presentation is the
   a) Sacrum
   b) Meutum
   c) Occiput
   d) Brow
3. The prelacunar stage of pregnancy happens on
   a) 3rd day of gestation
   b) 4th day of gestation
   c) 7th day of gestation
   d) 12th day of gestation
4. Pregnant women experience tingling/pricking sensation around the nipple between 3rd to 4th week of pregnancy as a result of
   a) Increased levels of estrogen in the blood
   b) Decreased levels of estrogen in the blood
   c) Increased levels of human gonadotrophic hormone
   d) Increased blood supply around the nipple area
5. Venereal disease research laboratory (VDRL) test is done to detect the presence of
   a) Genital vaginitis
   b) Syphilis
   c) Herpes simplex
   d) Gonorrhea
6. Cervical effacement refers to
   a) Dilatation of the cervix
   b) Inclusion of the cervix from the lower uterine segment
   c) Exclusion of the cervix from the lower uterine segment
   d) Inclusion of the upper segment of the uterus
7. In the normal labour process, the membranes rupture
   a) Just before the onset of the 1st stage of labour
   b) During the 2nd stage of labour
   c) When the midwife performs the first vaginal examination
   d) At the end of the 1st stage of labour

8. During labour, frequency, length and strength of contractions are monitored and recorded every
   a) Hour
   b) 15 minutes
   c) 30 minutes
   d) 45 minutes
9. The normal fetal heart rate during labour should range
   a) 110-120 beats per minute
   b) 110-160 beats per minute
   c) 140-150 beats per minute
   d) 100-180 beats per minute
10. During an internal rotation the presenting part rotates:
   a) 1/8 of a circle when it meets the pelvic floor
   b) ¼ of a circle when it meets the pelvic floor
   c) 1/5 of a circle when it meets the pelvic floor
   d) 1/6 of a circle when it meets the pelvic floor
11. Early clamping of the cord as part of active management of the 3rd stage of labour should be done in the first
   a) 5 minutes after birth
   b) 3 to 5 minutes after birth
   c) 30 seconds to 3 minutes after birth
   d) 1 minute to 4 minutes after birth
12. A diagnosis of retained placenta is made when
   a) Placenta remains undelivered 30 minutes after the baby's birth
   b) Part of the placenta goes missing
   c) Placenta remains undelivered 1 hour after the baby's birth
   d) Placenta remains undelivered 5 minutes after baby's birth
13. A midwife will diagnose occipital posterior position when; on abdominal examination
   a) The occiput is easily palpable at the fundus
   b) The head has already engaged
   c) There is a saucer shaped depression at the or near the umbilicus
   d) The occiput is easily palpable at the lower pole of the uterus
14. A complete breech is where by
   a) The breech presents with hips flexed and legs extended on the abdomen
   b) One or both feet present at the lower pole of the uterus
   c) One or both hips are extended with knees flexed
   d) Hips and knees are both flexed with feet tucked in beside the buttocks

15. The recommended calorie intake requirement for a woman in labour is:
    a) 500 Kcal per hour
    b) 60 Kcal per hour
    c) 100 Kcal per hour
    d) 121 Kcal per hour
16. The louvset's manoevre is used
    a) In the birth of extended legs
    b) In the birth of extended arms
    c) When there is delay of the birth of the head
    d) When the perineum is tight
17. A woman ruptures membranes and the midwife observes the cord lying outside the vaginal canal. The immediate action he/she will take is
    a) Cover the cord with a clean cloth
    b) Pack the vaginal canal with cotton wool
    c) Replace the cord gently back into the vaginal canal
    d) Instruct the woman to lie still in bed
18. Hypovolemic shock in a woman who is newly delivered is mostly caused by
    a) Severe loss of blood
    b) Vomiting
    c) Dehydration
    d) Inadequate nutrition
19. Assessment of blood pressure in a woman who has had a normal delivery in the last one hour is done
    a) To provide a baseline recording for monitoring her condition
    b) To check whether she is hypertensive
    c) To decide how to control a rise in the same
    d) To enable diagnosis of a heart condition
20. The placenta is fully functional by:
    a) 5th week of pregnancy
    b) 8th week of pregnancy
    c) 10th week of pregnancy
    d) 12th week of pregnancy
21. A midwife delivered a baby and after cutting the cord, rapidly dried the baby with a green towel. She/he was reducing the risk for
    a) Infection
    b) Hypothermia
    c) Common cold
    d) Respiratory failure

22. A woman came to the antenatal clinic for the 1st visit, her LMP was 13/11/2019. Her EDD will be on

a) 13/07/2020
b) 20/08/2020
c) 08/09/2020
d) 11/07/2020

23. The fetal membranes that make up the amniotic sac are the

a) Serous and chorion
b) Amnion and chorion
c) Serous and amnion
d) Chorion and mucous membrane

24. The leading direct cause of maternal mortality is

a) Sepsis
b) Hypertensive disorder
c) Hiv/Aids
d) Haemorrhage

25. The fetal heart rate is determined by

a) Palpation
b) Inspection
c) Percussion
d) Auscultation

26. In a fetal skull the normal diameter that engages in the mother's pelvis in vertex presentation is the

a) Occipital frontal
b) Bitemporal
c) Suboccipital bregmatic
d) Mental vertical

27. Indicators of an active phase of labour include

a) Frequent contractions occurring every 3-5 minutes, progressive dilatation of the cervix
b) Progressive dilatation of the cervix, contraction occurring every 5-10 minutes
c) A soft and pliable cervix, frequent contractions occurring every 3-5 minutes
d) Frequent contractions occurring every 10 minutes, progressive cervical dilatation

28. One of the following is the most important document in the management of labour

a) Admission notes
b) Carde
c) Birth notification
d) Patograph

29. Meconium stained amniotic fluid during labour when the presentation is cephalic is an indication of

a) Maternal distress
b) Abnormalities in the fetus
c) Fetal distress
d) Post datism

30. Artificial rupture of membranes is indicated when

a) Augmentation of labour is desired
b) Reduction of amniotic fluid in polyhydronious is desired
c) There is fetal distress

31. A fundus that is at the xiphisternum describes a pregnancy that is at gestation:

a) 36 weeks
b) 38 weeks
c) 30 weeks
d) 24 weeks

32. A cephalohaematoma forms when

a) Bleeding occurs at the base of the fetal skull
b) There is bleeding between the periosteum and the bones of the fetal skull
c) A vessel ruptures in the brain of a fetus labour
d) Fetal skull bones fracture during labour

33. An antenatal mother is supposed to attend the antenatal clinic for at least:

a) 8 visits before delivery
b) 6 visits before delivery
c) 5 visits before delivery
d) 4 visits before delivery

34. The drug of choice for routine anticonvulsant management of a woman with eclampsia is

a) Diazepam
b) Magnesium sulphate
c) Phenytoin
d) Hydralazine

35. The type of anemia common among pregnant women is includes:

a) Pernicious anaemia
b) Iron deficiency anaemia
c) Aplastic anaemia
d) Hemolytic anaemia

36. Maternal specific risks factors for pre-eclampsia include

a) Chromosomal abnormalities, extremes of age, nulliparity
b) Diabetes, extremes of age, renal disease
c) Urinary tract infections, chromosomal abnormalities, renal disease
d) Renal disease, urinary tract infection, hydatidiform mole

37. An individual birth plan includes
   a) Expected date of delivery, identification of a birth companion, health message on personal hygiene
   b) Identification of a possible blood donor, transport to be used before and after delivery, advise on good nutrition
   c) Identification of a birth companion, a possible blood donor, instructions on care of the nipple
   d) Expected date of delivery, identification of a possible blood donor, information on danger signs of the new born
38. One of the indications of vacuum extraction is
   a) A big baby
   b) Shoulder dystocia
   c) Prolonged 2nd stage of labour
   d) Absence of contraction
39. Causes of fetal distress during labour include
   a) Obstructed labour, cord prolapse, ante-partum haemorrhage
   b) Antepartum haemorrhage, fetal congenital malformation, malaria in pregnancy
   c) Antepartum haemorrhage, fetal congenital malformations, obstructed labour
   d) Utero-placental insufficiency, cord presentation, prolonged labour
40. A baby whose APGAR score is less than 7 after 5 minutes post delivery is managed for at least 24 hours in the
   a) High dependency unit
   b) Intension therapy unit
   c) Ward by the nurse
   d) New born unit

SECTION 2

SHORT ANSWER QUESTIONS (40 MARKS)

1. State FIVE precautions taken by a midwife to prevent fetal distress during labour (5 marks)
2. Explain the active management of the third stage of labour approach (AMTSL) (6 marks)
3. State FOUR causes of coagulopathy (clotting failure) in obstetrics (4 marks)
4. Outline FIVE aims of focused antenatal care (5 marks)
5. Explain THREE benefits of early initiation of breast feeding (6 marks)
6. State FOUR health messages you would give to a post-natal mother on nutrition (4 marks)

7. Outline FIVE risk factors for preterm labour (5 marks)
8. State FIVE signs of obstruction of labour on vaginal examination (5 marks)

SECTION 3

LONG ANSWER QUESTION (20 MARKS)

1. You have just delivered the baby and placenta and immediately the woman starts bleeding profusely.
   a) Define post-partum haemorrhage (1 mark)
   b) State FOUR possible causes of post-partum haemorrhage (4 marks)
   c) Describe the management of the woman until discharge from the labour ward (15 marks)